

LORATYN DRY SYRUP 1% w/w

VILORye-PC2

DESCRIPTION

White to off white raspberry flavoured powder. Raspberry flavour solution with white particles suspended upon reconstitution.

COMPOSITION

Each sachet contains: Loratadine Micronised 1.0% w/w

PHARMACODYNAMICS

Loratadine, the active ingredient in this medicine, is a tricyclic antihistamine with selective, peripheral H₁-receptor activity.

Loratadine has no clinically significant sedative or anticholinergic properties in the majority of the population and when used at the recommended dosage. During long-term treatment there were no clinically significant changes in vital signs, laboratory test values, physical examinations or electrocardiograms.

Loratadine has no significant H₂-receptor activity. It does not inhibit norepinephrine uptake and has practically no influence on cardiovascular function or on intrinsic cardiac pacemaker activity.

PHARMACOKINETICS

Absorption

Loratadine is rapidly and well-absorbed. Concomitant ingestion of food can delay slightly the absorption of loratadine but without influencing the clinical effect. The bioavailability parameters of loratadine and of the active metabolite are dose proportional.

Distribution

Loratadine is highly bound (97% to 99%) and its active metabolite moderately bound (73% to 76%) to plasma proteins. In healthy subjects, plasma distribution half-lives of loratadine and its active metabolite are approximately 1 and 2 hours, respectively.

Biotransformation

After oral administration, loratadine is rapidly and well absorbed and undergoes an extensive first pass metabolism, mainly by CYP3A4 and CYP2D6. The major metabolite-desloratadine (DL)- is pharmacologically active and responsible for a large part of the clinical effect. Loratadine and DL achieve maximum plasma concentrations (T_{max}) between 1-1.5 hours and 1.5-3.7 hours after administration, respectively.

Elimination

Approximately 40% of the dose is excreted in the urine and 42% in the faeces over a 10 day period and mainly in the form of conjugated metabolites. Approximately 27% of the dose is eliminated in the urine during the first 24 hours. Less than 1% of the active substance is excreted unchanged in active form, as loratadine or DL. The mean elimination half lives in healthy adult subjects were 8.4 hours (range=3 to 20 hours) for loratadine and 28 hours (range=8.8 to 92 hours for the major active metabolite).

Renal impairment

In patients with chronic renal impairment, both the AUC and peak plasma levels (C_{max}) increased for loratadine and its metabolite as compared to the AUCs and peak plasma levels (C_{max}) of patients with normal renal function. The mean elimination half-lives of loratadine and its metabolite were not significantly different from that observed in normal subjects. Haemodialysis does not have an effect on the pharmacokinetics of loratadine or its active metabolite in subjects with chronic renal impairment.

Hepatic impairment

In patients with chronic alcoholic liver disease, the AUC and peak plasma levels (C_{max}) of loratadine were double while the pharmacokinetic profile of the active metabolite was not significantly changed from that in patients with normal liver function. The elimination half-lives for loratadine and its metabolite were 24 hours and 37 hours, respectively, and increased with increasing severity of liver disease.

Elderly

The pharmacokinetic profile of loratadine and its active metabolite is comparable in healthy adult volunteers and in healthy geriatric volunteers.

Loratadine and its active metabolite are excreted in the breast milk of lactating women.

INDICATION

For relief of symptoms associated with allergic rhinitis, such as sneezing, nasal discharge (rhinorrhea) and itching, and ocular itching and burning. Nasal and ocular signs and symptoms are relieved rapidly after oral administration.

Also indicated for relief of symptoms and signs of chronic urticaria and other allergic dermatological disorders.

CONTRAINDICATIONS

Loratadine is contraindicated in patients who are hypersensitive to the active substance or to any of the excipients in these formulations.

PREGNANCY AND LACTATION

Pregnancy

A large amount of data on pregnant women indicates no malformative or feto/neonatal toxicity of loratadine. Animal studies do not indicate direct or indirect harmful effects with respect to reproductive toxicity. As a precautionary measure, it is preferable to avoid the use of this medicine during pregnancy.

Breast-feeding

Loratadine is excreted in breast milk, therefore the use of Loratadine is not recommended in breast-feeding women.

WARNING AND PRECAUTIONS

Loratadine should be administered with caution in patients with severe liver impairment. The administration of loratadine should be discontinued at least 48 hours before skin tests since antihistamines may prevent or reduce otherwise positive reactions to dermal reactivity index.

Effects on ability to drive and use machines

No impairment occurred in patients receiving loratadine. However, patients should be informed that very rarely some people experience drowsiness, which may affect their ability to drive or use machines.

DRUG INTERACTIONS

- When administered concomitantly with alcohol, this medicine has no potentiating effects as measured by psychomotor performance studies.
- Potential interaction may occur with all known inhibitors of CYP3A4 or CYP2D6 resulting in elevated levels of loratadine, which may cause an increase in adverse events.
- Increase in plasma concentrations of loratadine has been reported after concomitant use with ketoconazole, erythromycin, and cimetidine in controlled trials, but without clinically significant changes (including electrocardiographic).

MAIN SIDE/ ADVERSE EFFECTS

In a paediatric population children aged 2 through 12 years, common adverse reactions reported were headache, nervousness, and fatigue. The most frequent adverse reactions in adults and adolescents were somnolence, headache, increased appetite, and insomnia.

Other adverse reactions reported very rarely are listed below.

Immune system disorders:

Hypersensitivity reactions (including angioedema and anaphylaxis)

Nervous system disorders:

Dizziness, convulsion

Cardiac disorders:

Tachycardia, palpitation

Gastrointestinal disorders:

Nausea, dry mouth, gastritis

Hepato-biliary disorders:

Abnormal hepatic function

Skin and subcutaneous tissue disorders:

Rash, alopecia

General disorders and administration site conditions:

Fatigue

OVERDOSE AND TREATMENT

Overdosage with loratadine increase the occurrence of anticholinergic symptoms. Somnolence, tachycardia and headache have been reported with overdoses. In the event of overdose, general symptomatic and supportive measures are to be instituted and maintained for as long as necessary. Administration of activated charcoal as a slurry with water may be attempted. Gastric lavage may be considered. Loratadine is not removed by haemodialysis and it is not known if loratadine is removed by peritoneal dialysis. Medical monitoring of the patient is to be continued after emergency treatment.

DOSAGE AND ADMINISTRATION**Adults:**

One 10 mg dose as Loratadine (2 sachets) per day, administered orally in solution after meal. The dose is adjusted based on age and symptoms.

Children from the age of 3 to 7:

- Body weight < 30 kg: One 5 mg dose as Loratadine (1 sachet) per day, administered orally in solution after meal.
- Body weight > 30 kg: One 10 mg dose as Loratadine (2 sachets) per day, administered orally in solution after meal.

Children from 7 and older:

- Body weight < 30 kg: One 5 mg dose as Loratadine (1 sachet) per day, administered orally in solution after meal.
- Body weight > 30 kg: One 10 mg dose as Loratadine (2 sachets) per day, administered orally in solution after meal.

Patients with severe liver impairment should be administered a lower initial dose because they may have reduced clearance of loratadine; an initial dose of 5 mg or 5 ml once daily, or 10 mg or 10 ml every other day is recommended.

Directions for reconstitution:

Loratyn Dry Syrup 1% w/w is administered by reconstituting the powder with 150 ml to 200 ml water. One sachet of Loratyn Dry Syrup 1% w/w provides 5 mg of Loratadine upon reconstitution and it should be taken immediately after preparation.

Storage:

Store below 30°C. Keep in a dry place.

Presentation/ Packing:

30 sachets per box.

Product Registration Holder: HOVID Bhd.

121, Jalan Tunku Abdul Rahman (Jalan Kuala Kangsar),
30010 Ipoh, Perak, Malaysia.

Manufactured by: HOVID Bhd.

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Information date: June 2017